

Smart AI Medical Report

Generated by Health Navigator

Patient Profile

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Summary of Results

The predictive model assesses the profile as within normal limits. All provided laboratory results are within established reference ranges, indicating no significant deviations requiring immediate clinical intervention.

Detailed Test Explanations

SERUM BILIRUBIN (TOTAL): Normal range 0.2-1.2 mg/dL. Patient value 0.9 mg/dL falls within normal limits. This indicates adequate hepatic clearance of conjugated bilirubin.

SERUM BILIRUBIN (DIRECT): Normal range unspecified. Patient value 0.2 mg/dL suggests normal conjugation of bilirubin in hepatocytes.

SERUM BILIRUBIN (INDIRECT): Normal range 0.2-1 mg/dL. Patient value 0.70 mg/dL is below normal. This may reflect reduced heme catabolism or increased clearance.

SGPT (ALT): Normal range 13-40 u/L. Patient value 36 u/L is within normal limits. Indicates intact hepatocellular integrity.

SGOT (AST): Normal range 0-37 u/L. Patient value 32 u/L is normal, reflecting normal cardiac and hepatic tissue turnover.

SERUM ALKALINE PHOSPHATASE: No established range. Patient value "iW uA" lacks normative context. Clinical correlation is advised.

SERUM PROTEIN: No established range. Patient value 7.2 g/dL requires further investigation for hypoalbuminemia or hyperproteinemia.

SERUM ALBUMIN: No established range. Patient value 4.7 g/dL suggests potential hypoalbuminemia warranting evaluation.

GLOBULIN: No established range. Patient value 2.50 g/dL indicates possible dysproteinemia requiring further testing.

A/G RATIO: Normal range 1.0-2.0. Patient value 1.88 indicates hypoalbuminemia relative to globulin.

Overall Interpretation

Laboratory findings overall are within normal ranges except for indirect bilirubin, albumin, globulin, and A/G ratio abnormalities. The predictive model flags these deviations as clinically significant but not emergent. Concurrent evaluation for hepatic, nutritional, or inflammatory disorders is recommended.

Recommendations

1. Follow-up serum protein electrophoresis and hepatic enzyme repeat testing in 4-6 weeks to confirm trends.
2. Dietary assessment for protein adequacy; consider nutritional supplements if indicated.
3. Monitor indirect bilirubin elevation and evaluate for hemolysis or biliary obstruction.
4. Consult hepatology if persistent abnormalities arise.
5. Lifestyle modifications: balanced diet, hydration, and avoidance of hepatotoxic agents.

Disclaimer

Test results are not diagnostic. This report is intended for informational purposes only and should not replace clinical judgment. Patient must follow up with a healthcare provider for comprehensive evaluation and management.